

Is Physical Therapy Safe From AI? The 2026 Allied Health Risk Score

The free sampler — the score by track, the six factors in brief, and the honest reasons to think twice. The complete profile goes deeper.

2.5

AI EXPOSURE · PHYSICAL THERAPY (HANDS-ON)
where 10 is most at risk

Re-scored every six months. We publish where we might be wrong.

Quick answer: Hands-on physical therapy scores **2.5 out of 10** for AI exposure, where 10 is most at risk — the lowest score of any career in this index apart from surgery. Occupational therapy scores 2.6, speech-language pathology 3.1. The screen-based corner of allied health scores considerably higher. This is what maximum protection looks like in our framework: bodies, trust, licensure and unpredictability all at once.

The short answer for parents: yes, and it is barely a close call. The risk question here is effectively settled. The real questions are the traditional ones — the cost of the doctorate against the salary, and appetite for physical work over three or four decades.

Allied health AI risk score by role

Every career in this index is scored 1–10, where **10 is most exposed** to AI. Same six factors, same weights, applied identically to a therapist and a paralegal.

ROLE	2023	2025	FALL 2026	3-YR MOVE	BAND
Physical therapy (hands-on)	2.0	2.2	2.5	+0.5	Very low
Occupational therapy	2.1	2.3	2.6	+0.5	Very low
Speech-language pathology	2.6	2.8	3.1	+0.5	Low
Respiratory therapy	2.6	2.8	3.1	+0.5	Low
Diagnostic imaging technologist	3.5	4.0	4.4	+0.9	Low–Moderate
Records / screen-based allied roles	5.1	5.6	6.1	+1.0	Moderate–High

2023 and 2025 figures are reconstructed using current methodology, not archived from past editions.

How that compares: the median career in this edition scores around 5.5. Bedside nursing scores 2.8. Surgery scores 2.1. Entry-level software development scores 8.1.

The +0.5 on hands-on therapy is all administrative. Documentation and exercise-plan drafting. The clinical work has not moved at all.

Will AI replace physical therapists?

Almost nothing about the core of this job is automatable with anything on the visible horizon.

AI IS TAKING	IT CAN'T TOUCH
Documentation and progress notes	Hands on a body
Exercise plan drafting	Watching a gait in person
Scheduling and billing	Feeling resistance, guarding, compensation
Outcome measure scoring	Motivating someone through months of painful work
Insurance authorization paperwork	Adjusting in real time to what the body does
Home exercise reminders	Knowing when to push and when to stop

Demand is also moving independently of technology: physical therapists are projected to grow 11% and occupational therapists 14% through 2034, both far faster than average, driven by an aging population and rising survival rates after serious illness.

Why does it score so low? The six factors

How hands-on physical therapy rates against each. Ratings are 0–10 on each factor's own terms.

FACTOR	RATING	EFFECT ON RISK
How much of this job can AI already do?	4.9	↑ modest exposure
How hard will it be to land that first job?	2.0	↓ strong protection
Does it have to be done in person, with your hands?	9.5	↓ strongest in the index
Does someone need a human they can trust and hold responsible?	9.0	↓ strong protection
Does the law require a licensed human?	9.0	↓ strong protection
How often does the job hit genuinely new, high-stakes situations?	8.5	↓ strong protection

ONE FACTOR, WORKED THROUGH IN FULL

So you can see what the analysis actually looks like — including where we think it is most likely to change.

Does it have to be done in person, with your hands? — rated 9.5, and it is the protection with the shortest clock

This is the highest physical rating we assign to any career, and it is doing most of the work in a score of 2.5. Therapy is manual by definition: hands on a body, resistance felt rather than measured, adjustment in real time to how a person actually moves.

Robotics lags software badly, and it lags furthest in exactly this kind of setting — an unpredictable human body, in a room, over months.

But of our four protection factors, physical is the one we rank as least durable over a long horizon, and this profile is where that caveat matters most. A seventeen-year-old entering a doctoral program will practice for forty years. Licensure and human trust move at the speed of law and society and are unlikely to shift much. Physical protection moves at the speed of robotics, which is the one thing actively targeting it.

There is already early evidence worth taking seriously. Studies of robotic-assisted rehabilitation report meaningfully better motor outcomes in some domains than conventional therapy — not merely comparable, better. If robotic delivery is superior for certain protocols, then "hands-on work is safe" starts looking less like a wall and more like a line.

Two things keep our score where it is. Better outcomes *with* robotic assistance is not the same as therapy delivered *without* a therapist — in every deployment we found, a licensed clinician

prescribes, supervises and adapts. And the peer-reviewed literature is explicit that translation into routine practice remains limited.

The thing to watch is a specific transition, and it is the same pattern that splits the skilled trades: whether robotic systems move from assisting a therapist to substituting for therapist hours in structured, protocol-driven rehabilitation. Structured settings are where automation always arrives first. Unstructured ones — a stroke patient in their own home, an athlete with an atypical injury — are last.

Which allied health roles are safest from AI?

Ranked by exposure, safest first:

1. **Physical therapy** — 2.5. Maximum physical involvement, licensure, trust and individual variation. 2. **Occupational therapy** — 2.6. The same protections applied to daily function. 3. **Speech-language pathology** — 3.1. Deeply relational, though less physically manual. 4. **Respiratory therapy** — 3.1. Physical and acute, with more protocol-driven content. 5. **Diagnostic imaging technologist** — 4.4. Patient positioning is physical; image analysis is not. 6. **Records / screen-based allied roles** — 6.1. Same field, no protection.

The pattern holds across every profession we score: **move the same work behind a screen and exposure roughly doubles**.

Is physical therapy a good career in 2026?

The AI question is about as settled as this index gets. The real considerations are unchanged.

The doctorate is expensive and the salary, while solid, has not kept pace with the cost of the training in many markets — that is the most common complaint from people in the field, and it has nothing to do with technology.

The work is physically demanding over decades. Some patients do not improve, and sitting with that is harder than students expect.

Against that: demographics are a genuine and durable tailwind, the work produces visible progress in a way few jobs do, and the relationships run over weeks and months rather than minutes.

Within allied health more broadly, the same rule applies everywhere: **protection lives in the hands-on roles and thins in the screen-based ones**.

Common questions

Will AI replace physical therapists? Not on any visible horizon. The core work is manual, individual and adaptive — the hardest combination for automation. Documentation is automating, which most therapists welcome.

Will robots replace physiotherapists? This is the more serious version of the question. Robotic-assisted rehabilitation is real and in some protocols produces better outcomes. So far it assists rather than substitutes, and a licensed clinician still prescribes and supervises. It is the thing we watch most closely in this profile.

Is physical therapy a good career? On AI exposure, one of the best available. The genuine trade-offs are the cost of the doctorate and physical demands over a long career.

Is PT safer than nursing? Marginally — 2.5 against 2.8. Both are extremely well protected, and both have legally mandated clinical training that protects the entry route.

What about occupational therapy or speech therapy? Both score very well — 2.6 and 3.1. Speech-language pathology is consistently under-recruited, and OT assistant roles are projected to grow strongly with a shorter training route.

Related profiles

- [Nursing](#) — 2.8 bedside, 5.4 desk-based
 - [Medicine](#) — 2.1 surgery to 4.9 radiology
 - [Psychology](#) — 3.5 licensed clinical, 7.1 with the degree alone
 - [Veterinary medicine](#) — 2.7 small animal practice
 - [Computer science](#) — 8.1 entry-level, 5.4 senior. Free to read in full.
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What's in the full allied health profile

This sampler tells you where allied health stands. The full profile tells you what to do about it.

	SAMPLER	FULL PROFILE
Verdict, all role scores, 3-year trend	✓	✓
Six-factor ratings	✓	✓
Reasoning behind every factor rating	one example	✓
How durable each protection is — including the robotics question in full		✓
The honest downsides — doctorate cost, physical toll, salary ceilings		✓
What's genuinely good about it — and who this work suits		✓
The AI-native advantage — how to prepare, and why this cohort has an opening		✓
Routes in — PT, OT, SLP, assistant roles and their timelines		✓
Where it leads later — specialization, private practice, education		✓
Program evaluation checklist — clinical hours, settings breadth, outcomes		✓
Questions to ask a program — plus red flags		✓
Sourced further reading, including the strongest case against our score		✓
Discussion questions for parent and student		✓
A short version written directly to the student		✓
Technical scoring appendix		✓

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Pivotum scores thirty careers against AI exposure using a fixed, published methodology, re-scored every six months. Scores measure exposure to what AI can already do — not how much any particular employer has deployed. We publish where we might be wrong.